

Department of Pediatric Surgery
Discharge Summary

CR No.: anonymous	Name: anonymous	Age/Gender: 3.5 Yr/F
Admission No.: anonymous	Department Unit: Pediatric Surgery (Ped Sx Unit 1)	Consultant/Faculty: anonymous
Ward: Pedia Sx 3 C 4	Room/Bed: 3 C 4 Ward/ 3 C 4 Pae 13	Patient Category: General
Discharge Type: Normal Discharge	Father/Spouse Name: anonymous	Occupation:
Contact No.: anonymous	Address: anonymous	State/Country India
D.O.A. : 14-Mar-2023 12:51	D.O.D. : 04-Apr-2023	D.O. Print Report: 08-Feb-2026 15:42

FINAL DIAGNOSIS:

Congenital rectovestibular fistula;

OPERATION PERFORMED:

ASARP On 22-Mar-2023

SURGEONS: DR SONALI

ASSISTANT SURGEONS/SENIOR RESIDENTS: DR ASHWIN

TECHNICIANS/NURSES: NO BUSHRA

OPERATIVE FINDINGS: UNDER AAP, PATIENT IN SUPINE LITHOTOMY POSITION CPD DONE. URETHRA CATHETERISED WITH NO8 FOLEYS. SIZE OF SPHINCTER MUSCLE COMPLEX MARKED WITH MUSCLE STIMULATOR. RECTOVAGINAL FISTULA TO SML - RAQUET SHAPED INCISION MADE AND DEEPENED. MUSCLES WERE OPENED AND RECTUM IDENTIFIED. RECTUM WAS FIRST DISSECTED POSTERIORLY, THEN LATERALLY AND ANTERIORLY VAGINAL POSTERIOR WALL SEPARATED. ALL FIBROUS BANDS DIVIDED. RECTUM WAS FIXED WITH SPHINCTER MUSCLE COMPLEX. PERINEAL BODY CREATED. NEW ANUS CREATED OVER NO 12 DILATOR. RECTAL PACK KEPT.

CHIEF COMPLAINTS:

PASSING MECONIUM THROUGH VESTIBULE SINCE BIRTH

HISTORY OF PRESENT ILLNESS: PATIENT HAS HISTORY OF PASSING MECONIUM PER VESTIBULE FOR WHICH PATIENT WAS BROUGHT TO DISTRICT HOSPITAL AND REFERRED TO PAEDIATRIC SURGERY DEPARTMENT, AIIMS AFTER BIRTH. PATIENT WAS ADMITTED AFTER 2ND SAY OF LIFE. AND CONSERVATIVELY MANAGED AND DISCHARGE AND PRESENTLY NO FRESH COMPLAINTS, PLANNED FOR ASARP.

INVESTIGATION DETAILS:

Requisition Date	Test Name	Parameter Name	Result	Ref Range & Unit
16-Mar-2023 10:11	COMPLETE BLOOD COUNT (CBC)	Comments on PBS	Refer Reports*	-
		Absolute Basophil Count	0.03	0.02 - 0.10 x10 ³ /μL
		Absolute Eosinophil Count	0.25	0.1 - 1 x10 ³ /μL
		Absolute Lymphocyte Count	5.43	3.5 - 11 x10 ³ /μL
		Absolute Monocyte Count	0.11	0.1 - 1.2 x10 ³ /μL
		Absolute Neutrophil Count	3.22	1 - 7 x10 ³ /μL
		Basophil	0.3	00 - 01 %
		Eosinophils	2.8	01 - 06 %
		HCT	41.3	30 - 40 %



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		Hemoglobin	12.2	11.1 - 14.1 g/dl
		Lymphocytes	60.1	47 - 77 %
		MCH	22.0	24 - 30 pg
		MCHC	29.5	30 - 36 g/dL
		MCV	74.4	68 - 84 fL
		Monocytes	1.2	2 - 10 %
		Neutrophils	35.6	15 - 45 %
		Platelet Count	323	200 - 550 x10 ³ /μL
		RBC	5.55	3.9 - 5.3 x10 ⁶ /μL
		Red Cell Distribution Width (RDW CV)	14.7	30 - 36 %
		Total Leukocyte Count(TLC)	9.04	6 - 18 x10 ³ /μL
15-Mar-2023 11:30	BLOOD GROUPING	Blood Group	B	-
	ABO and RhD			
	Note	Refer Reports*		-
	RH Type	Positive		-

***Refer Lab Reports for details.**

HOSPITAL COURSE: PATIENT IS A DIAGNOSED CASE OF RECTOVESTIBULAR FISTULA. ADMITTED IN DEPARTMENT OF PEDITRIC SURGERY AND ALL ROUTIENE INVESTIGATION SNED UNDERWENT ANTERIOR SAGITTAL ANORECTOPLASTY (ASARP) ON 22/3/23. POST OP PERIOD WAS UNEVENTFUL. PATIENT RECOVERING WELL, HENCE BEING DISCHARGED WITH FOLLOWING ADVICE.

CONDITION AT DISCHARGE: PATIENT HAEMODYNAMICALLY STABLE, TOLERATING BREAST FEED, PASSING STOOLS, URINE, WOUND HEALING WELL.

ADVICE ON DISCHARGE:

SYRUP VITAMINE D 400 IU OD

BETADINE SYRINGING EVERY 2 HOURLY

SITZ BATH QID

MAINTAIN GOOD PERSONAL HYGIENE

CONTINUE BRESTFEED

REVIEW AFTER 1 WEEK

DISCHARGE BY : anonymous

DISCHARGE REMARKS :

FOLLOW UP ON : 11-Apr-2023

FOLLOW UP CONSULTANT / FACULTY : anonymous

FOLLOW UP ADVICE :

Summary Prepared By

anonymous

Summary Approved By

anonymous